and services. Third, foreign universities employ a lot of Indian faculty today. More Indian faculty would get the opportunity to work with foreign universities/institutions when they establish their branches here. This would help grow Indian academia.

India should permit 100 per cent FDI through automatic route (subject, of course, to technical affiliation, etc.) in the entire for-profit education space. We have a tremendous comparative advantage in education. Most of these foreign educational institutions are likely to be staffed by Indian teachers, researchers and technologists. Very quickly, foreign educational institutions and their competing Indian counterparts will make the country a global hub of education. From being a large importer of educational services, India can become a major exporter.

A Three-Pronged Policy Strategy in Health Will Work Wonders

Publicly funded, universal, good quality and easily accessible primary healthcare, a publicly funded health insurance programme to pay for secondary and tertiary healthcare services for the poor and a world-class private secondary and tertiary healthcare system for serving the better-off and global patients would make an ideal three-pronged strategy for India.

The Budget of the Department of Health and Family Welfare has fifty-one ‘heads’ of schemes.

These thinly spread micro-interventions do not serve India well. They should all be combined into two broad programmes to strengthen the primary healthcare system, and fund secondary and tertiary treatment for the poor. The objective of the first programme should be to establish, within five years, a comprehensive, good quality, functional, easily accessible and free primary healthcare system in the country. The objective of the second should be to pay for all the hospitalization needs of the poor and the vulnerable (not exceeding 10 crore genuinely poor families).